Tuia, Alfred, S 47Y , M

DOB: 12/02/1978 | **AC (MRN):** 35069

Preferred Name:
Sex: M
Previous Name:
Prefix:
Suffix:

Cell Phone: 702-680-9992 (Extended)
Home Phone: 702-680-9992
Work Phone:
Email: EPICRUSH808@GMAIL.COM (Web-Enabled)

Primary Insurance: Medicare
Race: Asian,White,Native Hawaiian or Other Pacific Islander
Advance Directive:

Ethnicity: Not Hispanic or Latino
Preferred Language: English
SSN: 500-82-5230

Confidential Patient: No
Deceased: No
Inactive: No

Sex Assigned at Birth:
Transgender:

Sexual Orientation:
Gender Identity:

PATIENT INFORMATION

Financial Information and Consents

Account Balance: 981.21
Patient Balance: 0.00
VFC Eligibility:
Mail Order Member ID:

Plan Type:
Don't Sent Statements: No
Exclude from Collections: No
Don't Add Financial Charge: No

Rx History Consent: U
Release of Info: Yes
Signature Date: Invalid date

Statuses

Student Status:
Is Native: No
Marital Status: Married

Exclude from Registry Search: No
Deceased: No
Deceased Date: N/A

Deceased Notes: N/A

Providers and Facilities

PCP:

Default Facility:

Rendering Provider: Lim, Marie Angeline Songco

Default Lab Company:

Referring Provider: Bowden, Dustin

Default DI Company:

External MRNs

Medical Record Number:

RESPONSIBLE PARTY

Name	Relation	Address	Cell Phone
Tuia, Alfred	Self - patient is the insured	4485 Pennwood Ave , 325 , Las Vegas , NV , 89102	702-680-9992

EMERGENCY CONTACTS

Name	Relation	Address	Preferred Phone	Guardian	HIPAA Permission
Tuia, April	Spouse		702-405-6345	No	

ADDRESSES

Address Type	Status	Address line1	Address line2	City	County	State	Zip Code	Country
Mailing	Current	4485 Pennwood Ave	325	Las Vegas		NV	89102	US
Street	Current							
Mailing	Previous	4485 Pennwood Ave	#325	Las Vegas		NV	89102	US

Residence Type:

Living Situation:

INSURANCES

Sequence/Type	Insurance Name	State	Subscriber No	Insured	Relation	Co-Pay	Group No
Primary Insurance	Medicare	ND	7G11WM3DR13	Tuia, Alfred, S	Self - patient is the insured		
Secondary Insurance	Nv Medicaid	NV	00001905731	Tuia, Alfred, S	Self - patient is the insured		

PHARMACIES

Name	Address	Type	Phone	Fax
SMITH'S PHARMACY 70600399	4840 W DESERT INN RD , LAS VEGAS , NV , 89102	Retail	702-248-1854	702-248-7042
SMITHS PHARMACY #706399	4840 W DESERT INN RD , LAS VEGAS , NV , 89102	Retail	702-248-1854	702-248-7042

CONTACTS

Name	Relation	Address	Preferred Phone	Emergency Contact	Guardian	HIPAA Permission
Tuia, April	Spouse		702-405-6345	Yes	No	

EMPLOYERS

Employment Status:

Employer	Employer Address	Job Title	Start Date	End Date	Status

CIRCLES OF CARE

Circle Name: ccmr_Team_1737500169445 **Circle Description:** created for ccmr

Circle Member Name	Member Type	Phone	Fax	Relationship

STRUCTURED DATA

Question Name	Value	Notes
1731073_OldAccountNO	423	423
1731073_OldAccountNO	423	423
1731073_OldAccountNO	423	423
1731073_OldAccountNO	423	423
1731073_OldAccountNO	423	423
1731073_OldAccountNO	423	423

TUIA, Alfred S **DOB:** 12/02/1978 (47 yo M) **Acc No.** 35069 **DOS:** 03/20/2026

Tuia, Alfred S

47 Y old Male, DOB: 12/02/1978
Account Number: 35069
4485 Pennwood Ave. 325, Las Vegas, NV-89102
Home: 702-680-9992
Guarantor: Tuia, Alfred S Insurance: Medicare Payer ID: 01312
Referring: Dustin Bowden
Appointment Facility: Buffalo - Center For Orthopedic, Pain, and Spine

03/20/2026 Carlos Nunez Cendan, FNP

Current Medications

Taking

- Mounjaro 10 MG/0.5ML Solution Auto-injector as directed Subcutaneous
- Farxiga 10 MG Tablet 1 tablet Orally Once a day
- DULoxetine HCl 30 MG Capsule Delayed Release Particles 1 capsule Orally Once a day
- BD Nano 2nd Gen Pen Needle 32GX 5/32" x 5/32" needle miscellaneous , Notes to Pharmacist: *Reorder from Medispan for eRx and Interaction Alerts*
- Metoprolol Tartrate 25 MG Tablet 1/2 tablet Oral Twice a day
- BD ULTRA-FINE PEN NEEDLE 32 gauge x 5/32" NEEDLE, DISPOSABLE MISCELLANEOUS , Notes to Pharmacist: *Reorder from Medispan for eRx and Interaction Alerts*
- Repatha 140 mg/mL Solution Prefilled Syringe Subcutaneous , Notes to Pharmacist: *Reorder from Medispan for eRx and Interaction Alerts*
- Famotidine 20 MG Tablet 1 tablet Oral Once a day
- Brilinta 60 MG Tablet 1 tablet Oral Twice a day
- Atorvastatin Calcium 80 MG Tablet 1 tablet Oral Once a day
- FLUoxetine HCl 10 MG Capsule Oral
- Fenofibrate 54 MG Tablet 1 tablet with food Oral Once a day
- Diclofenac Sodium 1 % Gel Apply 1 gram Externally Three times daily , Notes to Pharmacist: prn
- Allopurinol 100 MG Tablet 1 tablet Orally twice a day As needed
- oxyCODONE HCl 10 MG Tablet 1 tablet as needed Oral every 4- 6 hrs As needed

Reason for Appointment

1. Shoulder/back

History of Present Illness

Followup:

*

Interval HPI:

Patient is here today for post procedure follow up and medication refill.

Discussed MRI lumbar spine w/o contrast from SDMI performed on 10/13/25.

Discussed x-rays bilateral knees from SDMI performed on 10/13/25. It reveals bilateral knee osteoarthritis.

Patient underwent repeat bilateral L3-L5 radiofrequency ablation (RFA) on 07/15/25, with reported 50% pain relief.

Patient continues with Ortho and he will have a procedure to help with the frozen shoulder.

He has lost significant weight and reports it has helped a little bit with his lumbar and knee pain.

He states he had gel injections for bilateral knees with Ortho.

Patient states that L knee RFA is about 30% pain relief. The R knee, however, patient states the medial portion has improved but the lateral is still painful.

We received cardiac clearance from Dr. Tatiana Tsvetkova to hold Brilinta for one week prior to injections, low risk for cardiovascular complications.

Patient reports analgesic prescriptions are providing recognizably decrease in overall discomfort allowing for the maintenance of current level of activity.

TUIA, Alfred S **DOB:** 12/02/1978 (47 yo M) **Acc No.** 35069 **DOS:** 03/20/2026

- HYDROcodone-Acetaminophen 10-325 MG Tablet 1 tablet as needed Orally every 4-6 hrs As needed for severe pain

Past Medical History
- htn.
- dm.
- Medical History Verified.

Surgical History
- knee surgery MENISCUS 2004, 2007
- appendectomy 2015
- Heart stent x2 2019
- Surgical History Verified.

Family History
- Father: Headache , Hypertension , Kidney Problems , Liver Problems
- Mother: Arthritis , Cancer , Diabetes , Heart disease , Hypercholesterolemia , Seizures , Stroke
- Family History Verified.

Social History
Migrated Social History:
Migrated Social History: Tobacco history:Current every day smoker, , Is the patient capable of becoming pregnant?:No, , Drug Use:Denies any illegal drug use, , Employment:On disability, , Education level:High School Graduate, , Has the patient had 2 or more falls in the past year?:No, , Alcohol history:Never drinks alcohol, , Does the patient exercise?:No,.

Social History Verified.

Allergies
- Imitrex: anaphylaxis
- Dye, Red: anaphylaxis
- Allergies Verified.

Hospitalization/Major Diagnostic Procedure
- No Hospitalization Documented.
- Hospitalization Verified.

Review of Systems
General / Constitutional:
Patient denies Chills, Difficulty Sleeping, Easy Bruising, Excessive Sweating, Excessive Thirst, Fatigue, Fever, Low Sex Drive, Night Sweats, Tremors, Weight Gain, Weight Loss, Weakness.

Eyes:

His pain is 7/10, worst pain 9/10, least pain 5/10, average pain of last month 8/10. Worst pain bilateral knees and lower back . Denies radiating pain.

He describes his pain as stabbing/sharp. The pain is constant and worst during the day. General activity, normal work are negatively affected by his pain.

His pain has stayed the same since last visit. He has no new complaints.

He is stable on his current regimen. He denies adverse side effects from his regimen. His medications also improve his functioning and quality of life.

The patient presents for a refill of current pain medications. Patient states medications provide effective pain control and improve quality of life by at least 30%. The patient notes no serious side effects and feels the benefits of therapy outweigh the risks. Given the patient's stability on the current medication regimen, we did refill the medications today.

The patient's Nevada State Board of Pharmacy Controlled Substances Prescription Monitoring Program report was reviewed today and was consistent with the patient's medication history. Given the patient's stability on the current medication regimen, we did refill the medications without change today. Urine drug screen obtained from the patient was consistent with the patient's current medication regimen. The patient's urine drug screen testing remains consistent for the medications currently prescribed. The patient also has a signed opiate agreement on file. All questions regarding the medication were answered.

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MRI LUMBAR SPINE WITHOUT CONTRAST,Date of Service: 10/13/2025 -SDMI
FINDINGS:
Vertebral bodies are normal in height. The alignment has some mild retrolisthesis of L5 on S1. The bone marrow signal is normal. Spinal cord signal appears normal. The conus medullaris is normal in position. The visualized aorta is normal in caliber. The paraspinal soft tissues appear normal. T12-L1: The T12-L1 disc is normal in height and hydration. No disc herniation is seen. No neural compromise. L1-2: The L1-L2 disc is normal in height and hydration. No disc herniation is seen. No neural compromise. L2-3: The L2-3 disc is normal in height and hydration. No disc herniation is seen. No neural compromise. L3-4: The L3-L4 disc is normal in disc height. Some facet overgrowth is seen. No canal or foraminal stenosis is seen. L4-5: The L4-L5 disc is desiccated. A broad-based disc bulge abuts ventral thecal sac. AP thecal sac diameter measures 11 mm. Some facet overgrowth is visualized. Mild narrowing the bony foramen is seen. Compared with previous exam, appears with very little significant change. L5-S1: The L5-S1 disc is desiccated with disc height narrowing. Some mild retrolisthesis of L5 on S1 is seen.

Patient denies Blurry Vision, Changes in Vision, Loss of Vision.

ENT:
Patient denies Dental Problem, Difficulty Hearing, Earaches, Nosebleeds, Sore Throat, Ringing in Ears, Sinus Problem.

Cardiovascular:
Patient denies Bleeding Disorder, Chest Pain, Deep Vain Thombosis, Fainting, High Blood Pressure, Irregular Heartbeat, Lightheadedness, Shortness of Breath, Swelling in the feet.

Genitourinary:
Patient denies Abdominal Cramps, Decreased Urine Flow/Frequency/Volume, Flank Pain, Painful Urination.

Musculoskeletal:
Patient denies Joint Pain, Joint Stiffness, Neck Pain, Myalgia, Muscle Weakness,. Patient complains of Back Pain, Pain in Shoulder(s).

Neurologic:
Patient denies Headache, Dizziness, Numbness/Tingling, Instability when Walking, Tremors, Seizures.

Psychiatric:
Patient denies Depressed Mood, Anxiety, Stress Problems, Suicidal Thoughts, Suicidal Planning.

Small disc bulge is noted. Facet arthropathy is seen. There is some mild to moderate left greater than right foraminal narrowing. Standardized clinical grading of central canal stenosis is as follows: Normal is greater than or equal to 11mm; Mild 9 to 11mm; Mild to moderate 9mm; Moderate 7 to 9mm; Moderate to severe 7mm; Severe is less than 7mm.

IMPRESSION:
1. Disc height narrowing at L5-S1. Mild retrolisthesis of L5 on S1 is seen. Small disc bulge is seen. Facet arthropathy is seen. There is some mild to moderate left greater than right foraminal narrowing.
2. Disc desiccation, small disc bulge at L4-L5. Some facet overgrowth is seen. Mild narrowing the bony foramen is seen. Compared with previous exam, appears with very little significant change.
3. Some facet overgrowth at L3-L4 with no neural compromise.

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LEFT KNEE X-RAY, Date of Service: 10/13/2025 -SDMI
FINDINGS:
Minimal tricompartmental osteoarthritis. No acute fractures or dislocations. No joint effusion. Alignment is unremarkable. Incidental note of heterotopic ossification at the anterior tibial tubercle. Incidental note of enthesopathic changes at the posterior cortex of the proximal tibia.

IMPRESSION:
Minimal tricompartmental osteoarthritis.

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RIGHT KNEE X-RAY, Date of Service: 10/13/2025 -SDMI
FINDINGS:
Mild tricompartmental osteoarthritis. No acute fractures or dislocations. No joint effusion. Alignment is unremarkable. Incidental note of heterotopic ossification at the anterior tibial tubercle. Incidental note of enthesopathic changes at the posterior cortex of the proximal tibia. Soft tissues are unremarkable.

IMPRESSION:
Mild tricompartmental osteoarthritis.

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Depression screening:
PHQ-9 Little interest or pleasure in doing things Several days, Feeling down, depressed, or hopeless Not at all, Trouble falling or staying

asleep, or sleeping too much Nearly every day, Feeling tired or having little energy Not at all, Poor appetite or overeating Not at all, Feeling bad about yourself or that you are a failure, or have let yourself or your family down Not at all, Trouble concentrating on things, such as reading the newspaper or watching television Not at all, Moving or speaking so slowly that other people could have noticed; or the opposite, being so fidgety or restless that you have been moving around a lot more than usual Not at all, Thoughts that you would be better off dead or of hurting yourself in some way Not at all.

Vital Signs

BP: **116/66** mm Hg, HR: **100** /min, RR: **19** /min, Oxygen: **97** %, Wt: **215** lbs, Wt-kg: 97.52 kg, Ht: 70.00 in, Ht-cm: 177.8 cm, BMI: **30.85** Index, Pain scale: 7 1-10, Body Surface Area: 2.19.

Examination

CWPC Exam:

General appearance: alert, well-nourished and in no acute distress, Alert and Orientated x3, No acute distress.
Head: normocephalic, atraumatic.
Musculoskeletal: left shoulder active rom roughly 20 degrees with abduction. Passive to 30 degrees abduction. Tender throughout left shoulder.
Heart: no visible edema, no jugular venous distention.
Lungs: non labored breathing.

Cervical Spine:

Palpation: with diffuse tenderness over the paracervical muscles.
Range of motion: with decreased extension, with decreased flexion, with pain on movement.
Neck: reduced range of motion.

Back:

Palpation: paralumbar muscle tenderness with spasms, midline tenderness.
Range of motion: decreased full range of motion, decreased flexion, decreased extension, secondary to pain, secondary to stiffness.
Tests: positive straight leg raise, in seated position.

Assessments

1. Lumbar facet arthropathy - M47.816 (Primary)
2. Spondylosis without myelopathy or radiculopathy, lumbar region - M47.816 **Specify :** Src Problem: Facet arthropathy, lumbar
3. Pain in right shoulder - M25.511
4. Pain in right wrist - M25.531 **Specify :** Src Problem: Right wrist pain
5. Pain in right knee - M25.561
6. Bilateral primary osteoarthritis of knee - M17.0
7. Encounter for therapeutic drug level monitoring - Z51.81
8. Pain in unspecified knee - M25.569 **Specify :** Src Problem: Knee pain
9. Long term (current) use of opiate analgesic - Z79.891
10. Adhesive capsulitis of left shoulder - M75.02

47-year-old male with chronic bilateral knee pain. Pt is stable on current medication regimen with occasional knee injections. Suspect left shoulder pain is due to labral pathology, rotator cuff pathology, and adhesive capsulitis confirmed on MR L Shoulder. He has very limited exam. Unable to tolerate conservative measures. Will send to Ortho for further management.
Has current mental illness and is currently on disability followed by psychiatrists Chronic bilateral knee pain, reportedly unable to get knee replacement due to insurance

S/Pleft knee injection on 10/7/22 with 50% relief
S/P 3/6/24 bilateral knee genicular RFA. Patient reports pain relief of 40 % on the left knee and still with moderate pain on the right knee. Explained patient that best result is between 4-6 weeks after RFA-s/p bilateral knee"viscosupplementation injections" for bilateral knee DJD. Recommend doing these in a series of 3; 1 week apart

-Patient is status post 7/12/2023, 7/18/23, 8/1/23 Pt reports good relief with the injection/ his knee's don't swell as much. Left knee is doing better than the right
-s/p right genicular block on 9/6/22 with great relief nearing her more and more ease with walking and range of motion
s/p bilateral Knee RFA genicular nerve block - 30% pain relief on L knee. R knee medial part has improved but lateral part has not.
S/P Bilateral Lumbar MBB L3, L4, L5 10/16/24 - reports 80% pain relief for 2 weeks.
-S/P Bilateral Lumbar MBB L345 12/3/24 with 80% relief for at least the duration of the anesthetic
S/P 1/8/25 bilateral L3,L4,L5 RFA with 60% reduction pain for 6 months
S/P 07/15/25 bilateral L3,L4,L5 RFA with 50% reduction pain

Plan:
-Suspect non radiating low back pain is due to lumbar spondylosis confirmed on MR L Spine
-ORDERed referral to Dr. Cameron King with OIH for his left shoulder

-Ordered bilateral L5-S1 lumbar facet fusion. The patient has chronic radicular low back pain which has remained refractory to 6-12 months of conservative measures including medications, therapies, and spine interventions. Patient MRI reveals retrolisthesis L5 on S1. Disc desiccated with disc high narrowing and facet arthropathy.
Hx of coronary artery disease. Patient on Brilinta and Aspirin. Patient needs cardiologist clearance prior facet fusion. Clearance form printed and given to the patient
-Order flexion/extension x-rays lumbar. Rule out lumbar instability

-Pending of Cardiologist clearance, EKG, lab work
Medications
-Continue Hydrocodone/Acetaminophen 10/325 mg q 4-6 hours PRN for severe pain # 150 (for opioid rotation with Oxycodone)
-Hold Oxycodone 10mg q 4-6 hours PRN for severe pain #130

-D/C MS Contin 15mg QD
-Continue allopurinol 100 mg BID for possible gout in his right wrist; has been effective
-Colchicine was not effective. This was prescribed by his orthopedic
-Discussed potential referral to Rheumatology for further management of this medication. I discussed that we will not manage this medication moving forward but needs to be through PCP

-Enrolled to CCM program
-Previously ordered Back brace -46 inches. Awaiting to dispense.

-F/u 1 month for reassessment and medication refills

UDS 10/21/25 +opiate. Appropriate

Given the patient's history, physical exam, treatment plan, prescribed medications, and risk assessment plan I am requesting presumptive/standard urine drug testing to monitor for medication compliance and diversion. The urine drug testing for this patient is based on individual risk potential which I believe is in a low risk category for this patient with anticipated twice yearly screens.

Treatment

1. **Lumbar facet arthropathy**
   IMAGING: x ray : lumbar spine, flexion/extension views (Ordered for 03/20/2026)

   Notes: Nunes Cendan, Carlos 03/20/2026 08:27:36 AM PDT > Order flexion/extension x-rays lumbar. Rule out lumbar instability, spondylolisthesis, translation, angulation Nunes Cendan, Carlos 03/20/2026 08:27:56 AM PDT > send to SDMI

2. **Spondylosis without myelopathy or radiculopathy, lumbar region**

   Continue HYDROcodone-Acetaminophen Tablet, 10-325 MG, 1 tablet as needed, Orally, every 4-6 hrs As needed for severe pain, 30 days, 150, Refills 0
3. **Bilateral primary osteoarthritis of knee**

   Notes: I discussed the risks of opioid medications for pain management. These risks include addiction, substance abuse, medication tolerance, physical dependence, respiratory depression, constipation, nausea, and an opioid-induced hyperalgesic state (increase in pain from opioid medications). The patient has been warned to keep their medication in a safe place. Proper use, storage, and disposal of the medication was discussed. They have also been told not to drink alcohol while using the medication. The patient has been informed that The Center for Wellness and Pain Care of Las Vegas conforms to all of the recommendations of the Nevada Medical Board. Specific risks were discussed with the patient including not operating a vehicle while taking opioids because of increased risk of a motor vehicle accident. We discussed how

tolerance limits the efficacy of opioids for long-term pain management. The patient denies any abuse of the medications. The patient does not have any signs of opioid withdrawal or excessive sleepiness. The patient believes the medication is helping to control the pain. The patient has signed an opioid contract and states they are only taking the medication as prescribed. Discussed the common and serious side effects of this medication and the need to alert me if experiencing these or having any concerns about a possible medication effect. Common side-effects may include drowsiness, nausea, weight gain, imbalance, thinking abnormalities, seizures, depression/suicide and other side-effects. Advised the patient to also discuss possible side effects, contraindications, and interactions with their pharmacist.

Patient presents for a refill of current pain medications. Patient states medications provide effective pain control, and improve quality of life by at least 30%. Patient notes no serious side effects, and feels the benefits of therapy outweigh the risks. Given the patient's stability on the current medication regimen, we did refill the medications today. Patient's Nevada State Board of Pharmacy Controlled Substances Prescription Monitoring Program report was reviewed today. Patient will perform a urine drug screen today, with most recent prior UDS performed within the last three months. An opioid risk tool was also utilized to determine opioid overdose risk profile. Also, the appropriate use of Narcan was discussed and prescription was offered on today's visit if not already offered previously.

4. Encounter for therapeutic drug level monitoring

Notes: Clinical Info: Please note: Confirmatory tests are forwarded to a reference laboratory (Quest, LabCorp, CPL, or etc) and any associated expenses arising from such confirmatory test(s) are the responsibility of the patient. I certify, as the patient, that I have voluntarily provided a fresh and unadulterated specifmen for analytical testing. I have reviewed the information associated with this form and verified all information is accurate.

Visit Code

- 99214 OFFICE/OUTPATIENT VISIT EST.

Procedure Codes

- 1036F TOBACCO NON-USER.

This encounter has been **Marked as Done**

Follow Up

4 Weeks

Electronically signed by Provider: Carlos Nunes Cendan , FNP on 03/20/2026 at 05:15 PM PDT

Sign off status: Completed

TUIA, Alfred S **DOB:** 12/02/1978 (47 yo M) **Acc No.** 35069 **DOS:** 03/20/2026

Buffalo - Center For Orthopedic, Pain, and Spine
401 N. Buffalo Ste. 202
Las Vegas, NV 89145-0397
Tel: 702-476-9700
Fax: 702-476-9138

Progress Note: Carlos Nunez Cendan, FNP 03/20/2026

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